

Date: 01/01/2025 **Consultation Type:** Home Visit

S (Subjective): Visited at home today at the request of his sister, who has significant concerns about his mental state. Patient has been registered at the practice for 5 years.

History of Presenting Complaint: Patient (aged 71) describes a two-month history of developing persecutory beliefs centred on a former colleague named "Mike". He believes Mike is a "master of disguise" and is constantly following and monitoring him. He has identified several different people in the community (new postman, a lady on a bus, a coffee shop barista) as being Mike in disguise. These beliefs are fixed and held with absolute conviction. He has collected blurred photographs of strangers which he presents as "evidence" of this. As a result, he is now refusing to leave his house. His sister reports he has cut off contact with her, accusing her of "deceiving him". Neighbours have also raised concerns about a deterioration in his personal hygiene and an incident where he made an accusation against them, resulting in police being called.

Past Medical History:

- Depression (2020) - Treated with Sertraline 100mg for 18 months, treatment successful and discontinued.
- Head Injury (c. 2020) - Following an assault, required hospital admission.

Drug History:

- No current regular medications.
- No known drug allergies.

Social History: Lives alone in his own home. Now socially isolated and appears to be self-neglecting.

O (Objective): On examination, the patient is unkempt in appearance and visibly agitated and suspicious. He was reluctant to let me into the house.

Mental State Examination:

- **Appearance & Behaviour:** Unkempt, poor personal hygiene. Agitated, pacing intermittently. Maintained poor eye contact.
- **Speech:** Normal rate and volume, coherent.
- **Mood:** Anxious, agitated, and suspicious. Subjectively reports feeling "hunted".
- **Thought Content:** Preoccupied with a complex, fixed persecutory delusional system involving his former colleague. No suicidal or homicidal ideation expressed, but clear hostility towards his perceived persecutor.
- **Perception:** Denies any auditory or visual hallucinations. No evidence of perceptual disturbances during the consultation.

- **Cognition:** Alert and oriented to person, place, and time. Gross memory appears intact. Formal cognitive assessment was not performed.
- **Insight:** Absent. He has no insight into the delusional nature of his beliefs and rejects any alternative explanation.